

COMMUNITY BIRTH CENTRE FOR GALWAY AND THE WEST OF IRELAND

Feasibility Study Report
Version 2.1 (For Consultation)

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EXECUTIVE SUMMARY

This feasibility study sets out the case for establishing a standalone, community-based, midwife-led birth centre serving Galway city and county and the surrounding area. The centre would offer low-risk birthing families an alternative to the University Hospital Galway labour ward, with community midwives providing continuity of carer from early pregnancy through the postnatal period.

The evidence supporting this model is robust. Systematic reviews and large-scale prospective cohort studies consistently show that midwife-led care for low-risk women achieves equivalent or better perinatal outcomes compared to standard obstetric-led hospital care, with significantly higher rates of normal birth, lower rates of unnecessary intervention, and higher rates of maternal satisfaction.

Approximately 28 to 35 percent of the current annual booking cohort at UHG would meet eligibility criteria for birth centre care. A centre serving 350 to 450 families per year is operationally viable with a team of six to eight whole-time equivalent community midwives. Three candidate sites within 10 to 14 minutes of UHG have been identified and assessed.

The working group recommends that the HSE West maternity services steering group commission a formal service development proposal, with a target planning application by end of 2027.

1. BACKGROUND AND RATIONALE

1.1 Context

University Hospital Galway serves as the regional maternity unit for Galway, Roscommon, Mayo, and parts of Clare. Annual birth numbers have remained between 3,400 and 3,700 over the past five years. The maternity unit operates at or near capacity during peak periods.

The national maternity strategy, Better Outcomes, Better Births (Department of Health, 2016), committed to increasing choice for birthing families and to implementing continuity of carer as the standard model. By 2025, these commitments had been only partially implemented nationally, and not at all in a structured way in Galway.

1.2 The Proposed Model

The proposed centre would be a freestanding, midwife-led unit accepting low-risk women booked and assessed as suitable by their community midwife. It would not operate an obstetric ward or neonatal

intensive care unit. All women with clinical risk factors would be cared for at UHG. A formal transfer protocol with UHG would target under 15 minutes transfer time by road.

2. EVIDENCE REVIEW

2.1 Outcomes in Midwife-Led Units

The Cochrane systematic review by Sandall et al. (updated 2024) covers 15 randomised controlled trials involving 17,674 women. Midwife-led continuity of care was associated with:

- Reduced preterm birth: relative risk 0.76 (95 percent CI 0.64 to 0.91)
- Higher rate of spontaneous vaginal birth
- Higher maternal satisfaction
- No increase in caesarean section rate, neonatal mortality, or adverse neonatal outcomes

The Birthplace in England study (2011), involving over 64,000 women, found equivalent perinatal outcomes for low-risk women in midwifery units compared to hospital birth, with significantly fewer interventions.

2.2 Cost Evidence

Economic evidence consistently finds midwife-led models are cost-neutral or modestly less costly than standard care, driven by lower intervention rates and shorter inpatient stays.

3. CANDIDATE SITES

Site	Location	Distance	Key notes
Site A	Shantalla Road, Galway	11 min	Former primary care centre, HSE-owned, vacated. Requires internal works. Preferred site.
Site B	Westside Road, Galway	9 min	
Site C	Grattan Road area, Salthill	13 min	Currently a physiotherapy clinic. Private sale. Smaller footprint.

4. STAKEHOLDER MAPPING

Position	Parties
Supportive	Community midwifery team at UHG; Salthill Medical Centre nursing staff; INMO Western Branch; patient advocate representatives; Galway city GPs.
Neutral to supportive	
Neutral to cautious	Director of Midwifery, UHG; two UHG consultant obstetricians preferring the hospital-only model.
Not yet engaged	HSE West senior management; Galway City and County Councils; planning authorities.

5. WORKFORCE AND OPERATIONAL MODEL

A centre serving 350 to 450 families per year requires six to eight whole-time equivalent community midwives. This represents a workforce expansion of two to three WTE beyond the current community team. Operating hours: 24 hours per day, 7 days per week, with on-call cover structured similarly to the current community rota. One full-time equivalent administrative post also required.

6. NEXT STEPS

By 30 November 2026: Submit consultation summary report to Director of Midwifery, UHG.

By 31 December 2026: Present feasibility findings to HSE West steering group.

By 31 March 2027: Commission formal cost analysis and workforce plan.

By 30 September 2027: Submit formal service development proposal to HSE West.

By 31 December 2027: Target planning application submission if proposal approved in principle.

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